



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساميه محمد مديولي عمر
المحافظة : القاهرة الحي / المدينة : المقطم
رقم البطاقة : 25410280103061 رقم التليفون : 01001580439
العمر (تاريخ الميلاد) : 28-10-1954 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا أتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام أشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: سامية محمد مديوني عم
National ID: 25410280103061



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis

الوقت :

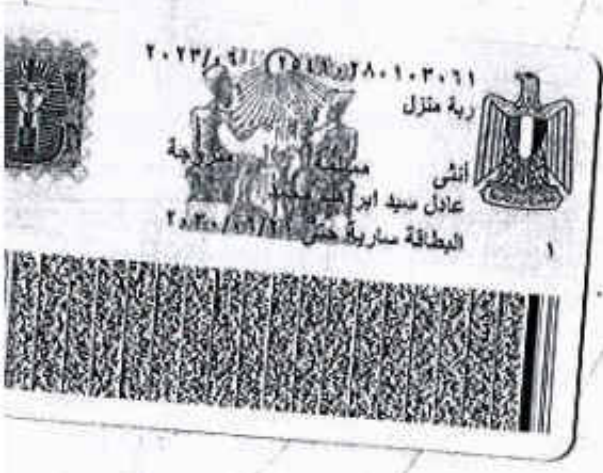
---:---

التاريخ :

05-07-2026

الطبيب المعالج :

د/شيماء لاشين



Patient Name : الأستاذة / سامية محمد مذبولى عمر :	Request No. : 13001838
Patient No. : 25410280103061	Request Date : 19/03/2022
Path No. : 771/2022	Verification Date : 09/04/2022
Sex : Female	Financial Category : اورام دار السلام - ميانء
Age : 65 Y	Department : Out Patient
Referreing Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT**Clinical Data:**

Right breast lesion

Nature of the specimen: Tru cut biopsy

Gross Examination:

Multiple greyish white tissue cores up to 1.5 cm , totally submitted

Microscopic examination:

• Histological type: Invasive duct carcinoma

• Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)

• Ductal carcinoma insitu : Focal

Diagnosis:

Right breast lesion, tru cut biopsy:

Invasive duct carcinoma grade II

IPT:

ER: Strong nuclear reaction in about 90% of tumour cells. Positive, score (8/8).

PR: Strong nuclear reaction in about 90% of tumour cells. Positive, score (8/8).

Her-2: Weak incomplete membrane reaction in more than 10% of tumour cells. Negative, score 1+

Ki67: 25-30% nuclear reaction.

Prof. Dr. Gamal Abd-El Aty

Dr. Esraa Ibrahim

Dr. Abdulrahman Ibrahim

عبد الرحمن ابراهيم
9/4

Verified by :

abdelrahman ibrahim

Printing Date : 09/04/2022 11:38:48

Page 1 of 1

Printed By : abdelrahman ibrahim

Name: ساميه محمد مديولى		ROUTE44
MR. : 4684		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض
رقم طبي:		بقسم:
ننتشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمناخه وجد انه		
The patient is a 71-year-old female with a history of right-sided breast cancer treated with right mastectomy and adjuvant chemotherapy, followed by radiation therapy to the breast and regional nodes. She has concurrent osteoarthritic changes of the acromioclavicular joint, supraspinatus tendinopathy, a partial thickness tear, and subcoracoid bursitis. Imaging of the cervical spine in October 2025 revealed vertebral collapse with abnormal bone marrow signal at C4, C5, and C6, along with disc bulge. A recent breast ultrasound performed on 8 March 2026 demonstrated a free femoral calcium deposit measuring 1 × 1 × 90 mm, described as "one alpha." The patient is currently on a regimen of femara calcium 1 × 1 × 90 mg, with an additional calcium 1 × 1 × 90 mg supplement. She has declined a bone scan and is scheduled for her next clinic visit on 26 August 2026. The patient remains under observation for bone health and oncologic surveillance.		
تحريرا في ,		
التاريخ :		19-09-2026
الوقت :		نهى سعيد على محمد
الطبيب المعالج :		بعتمد ,



Patient Name	: الأستاذة / سامية محمد مديولي عمر	Request No.	: 13001838
Patient No.	: 25410280103061	Request Date	: 19/03/2022
Path No.	: 771/2022	Verification Date	: 06/04/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 65 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:

Right breast lesion

Nature of the specimen: Tru cut biopsy

Gross Examination:

Multiple greyish white tissue cores up to 1.5 cm , totally submitted

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma insitu : Focal

Diagnosis:

Right breast lesion , tru cut biopsy

Invasive duct carcinoma , grade II

For IPT

Prof.Gamal Abdelatty
Dr.Esraa Ibrahim

Verified by :

dr.esraa magdy

Printing Date : 09/04/2022 09:15:13

Page 1 of 1

Printing By : .marwa mohamed youss



Name: Samya Mohamed

Date: 18-06-2022

NECK VEINS DOPPLER REPORT

Right side:

- ☐ The internal jugular, innominate, & subclavian vein are patent, compressible with normal color-flow pattern and monophasic Doppler spectral waveform, exhibiting respiratory phasicity and preserved atrial pulsations.

Left side:

- ☐ The internal jugular, innominate, & subclavian vein are patent, compressible with normal color-flow pattern and monophasic Doppler spectral waveform, exhibiting respiratory phasicity and preserved atrial pulsations.

CONCLUSION

- No central venous thrombosis.

Dr. Sherif Mostafa, M. Sc.



Patient Name: Samya Mohamed Madbouly.

Date: 19/7/2022

^{99m}Tc - ^{99m}Tc -MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- No definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for the referral

Nuclear Medicine Unit

DR. Sherin Mahdy, MD

Name : Samia Mohammed Date : 10/04/2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Right breast asymmetrical density.
- Right axillary level I and II enlarged lymph nodes the largest measures 2.3 X 1.4 cm.
- Left lower lung lobe calcified nodule mostly representing post granulomatous sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.
- Apertous lobe and fissure.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renal.
- Strength filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. Intramedullary skeleton:

- Bilateral acetabular sclerotic foci mostly representing bone islands.
- L1 lytic lesion measuring 1.5 cm with vertical striation likely osseous hemangioma, no collapse or soft tissue component.

OPINION:

- ***Reported case of breast carcinoma.***
- ***No definite hepatic or pulmonary metastatic deposits.***

Dr. Mohammed Mohsen, MSc.

Dr. M. Gomaa, M.D.



مركز النيل للأشعة و التحاليل

Name : Samia Mohammed Madbouli
Branch : Maadi
Ref.Physician Prof : Goustav Roussey Egypt

Date : 04-09-2026 09:18
Medical No. : 454158
تعليق مستشفى : Ins.Company :

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 72-year-old female with left breast carcinoma, status post mastectomy. Presents with generalized bony aches. Referred for follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Increased tracer uptake is noted at the region of cervical spine, for clinical/radiological correlation.
- A small focal area of uptake is noted at the right aspect of L3, likely facet joint arthropathy.
- Mild symmetrical uptake is noted in both knees, slightly more pronounced on the right, consistent with degenerative changes.
- The remainder of the axial and appendicular skeleton shows physiological tracer distribution without focal osteoblastic lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- No scintigraphic evidence of skeletal metastases.
- Clinical correlation and targeted imaging of the indeterminate uptake at cervical spine is advised if symptoms persist.
- Follow up imaging as clinically indicated.

Dr. Alaa Nagi MD

Nuclear Medicine

N I L E D I A G N O S T I C C E N T E R



Patient Name: Samia Mohamed Madboli Omar	Date : 07/04/2025
Exam : right upper limb venous doppler	Age : 70 Year(s)
Medical ID: 942/2022	Pat. ID: 13798

Duplex Examination of the arterial&venous system of the right upper limb revealed: -

Arterial system:

- > *Patent arterial system of the examined upper limb including subclavian, axillary, brachial, radial & ulnar arteries with normal intimal thickness.*
- > *Preserved arterial triphasic waveform configuration with average flow velocities of the examined arterial tree.*
- > *No evidence of arterial stenosis or occlusion seen.*

Venous system:

- > *The subclavian, axillary, radial & ulnar veins of the examined upper limb all are patent, totally compressible with normal color filling & preserved respiratory variation.*
- > *No evidence of D.V.T*
- > *patent basilic and cephalic vein along their whole course, no evidence of superficial thrombophlebitis at time of scan*

OPINION:-

No evidence of D.V.T or superficial thrombophlebitis at time of scan

No evidence of arterial stenosis or occlusion seen

Signature Dr . Nermien Ramnadan
07/04/2025 12:16 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
مركز الأبحاث الطبية المتخصصة
مستشفى أورام دار السلام
الدارة الإشعاعية

Patient Name: Samia Mohamed Madboli Omar	Date : 03/02/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 69 Year(s)
Medical ID: 942/2022	

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM in

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Dr.Yomna Tarek,M.Sc.

Not Approved

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
مركز المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الطابق الرابع

Patient Name: Samia Mohamed Madboli Omar	Date : 08/03/2025
Exam : MAMMOGRAPHY BOTH	Age : 70 Year(s)
Medical ID: 942/2022	Pat. ID: 13798

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

08/03/2025 12:55 PM

09/20/2026

MDT FORM

Name:	Samia Mohamed Madbouly *	MRN:	4684
Age:	72 Y	ID:	25410280103061
Menopausal Sta		Phone:	01001580439
NRS SCORE:	PS ECOG 1-2	Marital Status:	
BMI:		Family History:	
CO Morbidity:	free	Reason Of MDT:	met

Clinical Data:

R

BX: IDC GU

ER: S/S **PR:** q/s **HER2:** -ve **KI67:** 40%

Sonomammography:

Mets Workup:

History:

female pt 70 yrs ER , PR +ve , her2 - ve
underwent Rt MRM + SLNB (5-2022) >> pT2 N3 adJ LXT , endoxan >> PORT
>> on AI since 2023 till now
recently : Bone scan (8-9-2026) >> free

cervical MRI (19-8-2026) >> C2 C3 C4 vertebral bodies with cord indentation
at C4 , intraspinal soft tissue mass due to neoplastic lesion

for urgent palliative RTh to cervical spine

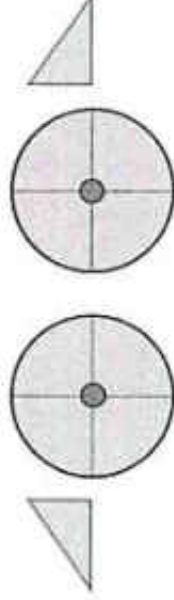
CT CAP >> R

Recommendations:

MDT Outcome & Plan

for faslodex /ibrance
ZOMETA EVERY 3MS AFTER DENTAL ASSESSMENT
urgent palliative rth

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. منال حوز	د. احمد لطيف				

Faath Islamic Center

C.T. - M.R.I

U.S. - Duppler - E.C.G. Stress

E.E.G - E.M.G - Mamm - Eye Unit



الجمعية الشرعية

مركز الفتح الإسلامي

أشعة مقطعية بالمبيوتر - الرنين المغناطيسي
موجات صوتية - دوتلر ملون - رسم قلب بالحجوة
رسم مخ - رسم عضلات - فحص لدى - وحدة عيون

التاريخ: ٢٠٢٦/٨/١٩

اسم المريض: سامية محمد مديوني

M.R.I. Of The Cervical Spine

Technique:

- Sagittal T1 & T2 WIs
- Axial T2* (G.R.I.E.) WIs

Findings:

- Abnormal marrow signal intensity of C2, C3 and C4 vertebral bodies displaying low T1 and low and intermediate T2 signal intensity. Relatively decreased height of C4 vertebral body and rotation of its posterior inferior aspect into the spinal canal with indentation of the subjacent theca and cord.
- Paravertebral soft tissue mass suspected intra-spinal epidural mass are seen at C3/C4 level of low T1 and non-homogenous high T2 signal intensity.
- The cervical spinal cord shows no masses or areas of abnormal signal intensity.
- Spondylotic changes are established with marginal osteophytic formation.
- Straightening of the cervical curve is noted.

Opinion:

- Changes seen at C2, C3 and C4 vertebral bodies with cord indentation at C4 level and paravertebral and ? intraspinal epidural soft tissue mass possibly due to neoplastic lesion, for contrast-enhanced M.R.I. study/CT scan for the cervical spine.
- Cervical spondylosis.

YOURS SINCERELY,

Prof. Dr. Mohy El-Hennawy

PROF. OF RADIO-DIAGNOSIS
AL-AZHAR UNIVERSITY



בית המדרש
החדש
של
מבשר

c.c/3/ev 5th, 1st

aspirine //
allergic to Sulfa //
Clinical Data

Recommendations:

upfront HRH, BCs is not Valid (2 chances with an area of indeterminate pathology & small breast size) $\pm$ SLN according to result of target axillary US. $\rightarrow$ ALND

12-14-11 SLN biopsy

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
			



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستقامة / ساليه محمد بنويحيى عسر
Patient No.	: 25410280103061
Path No.	: 1270/2022
Sex	: Female
Age	: 65 Y
Referring Doc :	
Request No.	: 13003405
Request Date	: 15/05/2022
Verification Date	: 28/05/2022
Financial Category	: اورام نادر السلام - مبالغه
Department	: Out Patient
Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Rt breast cancer

Nature of the specimen:

Rt MRM + SLNB

Gross Examination:

Rt MRM spicemen measured 21x13x6 cm with covering skin ellipse measured 18x13 cm

- Tumor extent

o Number: An ill defined greyish white&hemorrhagic firm mass

Size : 4x3x2 cm

o Skin involvement: Free nipple & areola grossly

- Distance from margins: 0.4cm from deep margin

- Lymph nodes number and size:

1-SLNs : 6 nodes up to 2x1 cm

2-ALNs: 14 nodes up to 2x1.5 cm

- Frozen section:

-SLNs : (6/6) with extracapsular invasion

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Angioinvasion: present
- Ductal carcinoma in situ: 10% comedo and non-comedo types
 - o Skin involvement: Free nipple & areola
- Lymph node involvement: - Positive (6/6) SLNs with extracapsular extension.
- Positive (14/14) ALNs with extracapsular extension.
- Total nodal status= Positive (20/20) LNs with extracapsular extension.

Verified by :

dr.menna atef

Request No.	: 13003405
Request Date	: 15/05/2022

Request No. : 15/05/2022
Request Date : 09/05/2022

Verification Date : 28/05/2022
اورام دار السلام - اسلام آباد

Financial Category:
: Out Patient

Registered by : marwa

Pathology Report

- Diagnosis:
Rt breast cancer, Rt MRM + SLNB:
• Invasive duct carcinoma Grade II with 10% Ductal carcinoma in situ, comedo and non-comedo types,
• Positive Angioinvasion and perineural invasion
o Free nipple & areola
• Positive (6/6) SLNs with extracapsular extension.
• Positive (14/14) ALNs with extracapsular extension.
- Total nodal status= Positive (20/20) Lymph Nodes with extracapsular extension.
• Free deep margin
• Stage: pT2, pN3

Dr. Esraa Magdy

Printing By: Ali mostafa

Page 2 of 2

Revised 26th, 1987. First Paper Published May 1987. 2200 Corniche, Silver Et Giza, Cairo, Egypt.

IMMUNOHISTOCHEMISTRY REPORT

Bloom: Na (6) - First Floor - Resident's Room; Title Certificate - Silver Et. Quintana - C. Alva

Request No. : 13001838

Request Date : 19/03/2022

Verification Date : 09/04/2022

Department : Out Patient

IHC No. 11

Registered by : marwa mohamed yousse

Right breast lesion

Gross Examination:

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+1)
- Ductal carcinoma insitu : Focal

Right breast lesion, tru cut biopsy:

invasive duct carcinoma grade II

IPt;

ER: Strong nuclear reaction in about 90% of tumour cells. Positive, score (8/8).
PR: Strong nuclear reaction in about 90% of tumour cells. Positive, score (8/8).

Her-2: Weak incomplete membrane reaction in more than 10% of tumour cells. Negative, score 1+

Ki67: 25-30% nuclear reaction.

Prof. Dr. Gamal Abd-El Atty

Dr. Esraa Ibrahim

Dr. Abdulrahman Ibrahim

3/4

Verified by:

abdelrahman ibrahim

Printing Date : 09/04/2022 11:38:48

Page 1 of 1

Printed By : abdelrahman ibrahim

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Date : 03/02/2024	Patient Name: Samia Mohamed Madboli Omar
Age : 69 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
	Medical ID: 932/2022

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

- Enlarged measures about 19 cm in MCL with homogenous echopattern
- No intra hepatic biliary radicle dilatation or focal lesions.
- Portal vein is patent

Gall Bladder:-

- Average distention with normal wall thickness with no stones.
- CBD is not dilated.

Spleen:-

- Average in size measures about cm displaying homogenous echopattern, no focal lesions.

Kidneys:-

- Are of normal size, well differentiated & no gross stones or back pressure changes or space occupying lesion could be seen.
- Para-aortic and pancreatic area: are concealed by gases.
- No ascites or free fluid or collection seen .

Urinary Bladder:-

- Average filling with no stones or focal lesion seen

Opinion:

- Mild hepatomegaly with no specific pathology.

Best regards
Dr/Shahd M. Fouad,Msc

أ. د. أيما دعبس
أخصائية أشعة

Patient name: Samya Mohamed MAdboly

Date: 12.3.2022

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Report:

- Right Breast:
 - predominantly fatty (ACR A)
 - Focal asymmetric density is seen in the right UOQ and LOQ with partially obscured high density lesion, pleomorphic calcifications adjacent spicules and skin dimpling covering an area measures 4 x 8 cm in MLO view seen by U/S at 8 o'clock measuring about 3 x 5 cm with overlying skin edema and dimpling
 - indeterminate enlarged level I axillary LN with cortical thickness measures 0.6 cm
 - No pathological enlarged infra or supraclavicular lymph nodes.
- left Breast:
 - predominantly fatty (ACR A)
 - benign calcifications.
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Right breast malignant mass with ipsilateral indeterminate axillary lymphadenopathy (BIRADS 5) for histopathological examination .
- Benign left breast findings(BIRADS 2)

Best regards,

Dr. Aya Deabes, MSc.



Name : Samia Mohammed Date : 10/04/2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:-

1. Chest:

- Right breast asymmetrical density.
- Right axillary level I and II enlarged lymph nodes the largest measures 2.3 X 1.4 cm.
- Left lower lung lobe calcified nodule mostly representing post granulomatous sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.
- Azygous lobe and fissure.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

Patient Name: Samya Mohamed Mabouly.

Date: 19/7/2022

^{99m}Tc-^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- No definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for the referral

Nuclear Medicine Unit

DR . Sherin Mabdy, MD



Name: Samya Mohamed

Date: 18-06-2022

NECK VEINS DOPPLER REPORT

Right side:

- ☐ The internal jugular, innominate, & subclavian vein are patent, compressible with normal color-flow pattern and monophasic Doppler spectral waveform, exhibiting respiratory phasicity and preserved atrial pulsations.

Left side:

- ☐ The internal jugular, innominate, & subclavian vein are patent, compressible with normal color-flow pattern and monophasic Doppler spectral waveform, exhibiting respiratory phasicity and preserved atrial pulsations.

CONCLUSION

- No central venous thrombosis.

Dr. Sherif Mostafa, M. Sc.



Name : Samia Mohammed Date : 10/04/2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:-**

1. Chest:

- Right breast asymmetrical density.
- Right axillary level I and II enlarged lymph nodes the largest measures 2.3 X 1.4 cm.
- Left lower lung lobe calcified nodule mostly representing post granulomatous sequel.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.
- Azygous lobe and fissure.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



4150

وزارة الصحة والسكان
المركز الوطني للأمراض القلبية المتخصصة
مستشفى فورام دار السلام

وزارة الصحة والسكان
امانة المركز الطبي المتخصصة
مركز فورام دار السلام

Dr. Mohamed El

Report

①

اسم المريض والسكن
رقم الملف الطبي
نوع التوجيه
تاريخ اعداد الوجهه
وقت اعداد الوجهه

EF	64	55-75%	PWT	1.0	0.6-1.1cm
LVEDD	5.6	3.5-5.8cm	AOR	3.0	2-3.7cm
LVEDD	3.6	2-4cm	LA	3.8	2-4cm
FS	35	25-45%	RV	ref	0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO CUSP	ref	1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Aortic root dimensions

Mitral Valve Tricuspid valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

Dr. Mohamed Anwar

Misc cardiology

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
مركز المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الطابق الرابع

Patient Name: Samia Mohamed Madboli Omar	Date : 08/03/2025
Exam : MAMMOGRAPHY BOTH	Age : 70 Year(s)
Medical ID: 942/2022	Pat. ID: 13798

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

08/03/2025 12:55 PM